Prodigy Anesthesia Quick Review

Endocarditis

What is the most common pathogen responsible for bacterial endocarditis?	Viridans streptococci (alpha-hemolytic streptococci) are the most common followed by staphylococcus aureus and enterococcus. *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 237.*
How does the incidence and severity of endocarditis differ when it affects the right-sided heart valves compared to the left-sided heart valves?	In the general population, left-sided endocarditis is more common and typically occurs as the result of bacteremia and carries a higher mortality rate than right-sided endocarditis. In injection drug users, however, right-sided endocarditis is more prevalent. *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 318.*
How are drug and alcohol abuse related to infective endocarditis?	A common complication of injection drug abuse is infective endocarditis. The frequency of injection and the concomitant presence of HIV are factors that increase the risk of endocarditis dramatically. The mortality rate among injection drug abusers who develop endocarditis is about 35% and increases in immunocompromised patients. Strangely, a concomitant diagnosis of alcohol abuse appears to decrease the risk of endocarditis in this population. *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 318.*
What types of procedures warrant antibiotic prophylaxis in patients at risk for endocarditis?	Prophylaxis is now recommended for at-risk patients undergoing urinary procedures when a concurrent bacterial urinary infection. It is not recommended for upper or lower gastrointestinal endoscopic procedures. *Miller RD. Miller's Anesthesia. 8th ed. Philadelphia: Elsevier Saunders; 2015: 1104.*
What types of patients are considered at-risk for infectious endocarditis?	Patients who have a previous history of infective endocarditis, have unrepaired cyanotic defects, patients within 6 months of repair of a congenital heart defect with a prosthetic device, patients with residual defects following repair of a congenital heart condition, or patients with valvulopathy that develops following valve transplantation. *Miller RD. Miller's Anesthesia. 8th ed. Philadelphia: Elsevier Saunders; 2015: 1104.*

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